

PROMOTION PHYSICAL THERAPY

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PHYSICAL THERAPY EVALUATION

PATIENT INFORMATION

Patient:	Maria Elena Santos	DOB:	03/14/1988
Eval Date:	February 19, 2026	Diagnosis:	Lumbar strain,
Referring MD:	Kevin Park, MD		Post-concussion
Therapist:	Jennifer Martinez, DPT	Insurance:	Hartford WC

HISTORY

35-year-old female carpenter referred for physical therapy following workplace injury on 02/08/2026. Patient fell approximately 15 feet from scaffolding, sustaining right distal radius fracture (currently in cast), concussion, and lumbar strain with MRI-confirmed L4-L5 disc herniation.

Current Complaints:

- Low back pain rated 6/10, constant, aching quality
- Pain radiates to left buttock and posterior thigh
- Difficulty sitting for more than 15-20 minutes
- Difficulty sleeping due to back pain
- Stiffness in the morning
- Residual headaches and dizziness (improving)

OBJECTIVE FINDINGS

Posture: Guarded posture with slight forward lean. Decreased lumbar lordosis.

Gait: Antalgic gait pattern with shortened stride length bilaterally.

Lumbar Range of Motion:	Motion	Measured	Normal
	Flexion	40°	80°
	Extension	10°	30°
	Left Lateral Flexion	15°	35°
	Right Lateral Flexion	18°	35°
	Left Rotation	20°	45°
	Right Rotation	22°	45°

Palpation: Tenderness and spasm of bilateral paraspinals L3-S1, more pronounced on left. Tenderness over L4-L5 spinous processes.

Special Tests:

- Straight Leg Raise: Positive on left at 45° (reproduces radicular symptoms)
- Slump Test: Positive on left
- Prone Instability Test: Negative
- FABER: Negative bilaterally

Neurological:

- Sensation: Decreased to light touch left L5 dermatome (lateral leg, dorsum of foot)
- Reflexes: 2+ bilateral patellar and Achilles

- Motor: 4/5 left great toe extension (EHL weakness)

Functional Assessment:

- Oswestry Disability Index: 48% (Severe disability)
- Unable to lift more than 5 lbs from floor
- Unable to sit more than 20 minutes
- Unable to stand more than 15 minutes

ASSESSMENT

1. Lumbar disc herniation L4-L5 with left L5 radiculopathy
2. Lumbar paraspinal muscle strain/spasm
3. Decreased lumbar mobility and core stability
4. Functional limitations affecting ADLs and work capacity

TREATMENT PLAN

Frequency: 3x per week for 12 weeks (36 visits authorized)

Goals (12 weeks):

1. Reduce pain to 2/10 or less
2. Restore lumbar ROM to at least 75% of normal
3. Improve Oswestry score to below 20% (minimal disability)
4. Return to modified work duties
5. Independent with home exercise program

Treatment Interventions:

- Manual therapy: Soft tissue mobilization, joint mobilization
- Therapeutic exercises: Core stabilization, McKenzie approach
- Modalities: Heat, electrical stimulation for pain/spasm
- Neuromuscular re-education
- Patient education: Body mechanics, posture, activity modification
- Progressive strengthening program

PROGRESS NOTE - 6 WEEK UPDATE

Date: April 1, 2026 | Visit #18

SUBJECTIVE

Patient reports steady improvement. Back pain decreased to 3-4/10. Radiating leg symptoms have significantly improved - now only occasional tingling with prolonged sitting. Sleeping better. Headaches have resolved. Right wrist cast was removed and she has begun occupational therapy for wrist as well.

OBJECTIVE

	Motion	Initial	Current	Normal
Lumbar ROM (Re-assessment):	Flexion	40°	65°	80°
	Extension	10°	22°	30°
	L Lateral Flex	15°	28°	35°

Special Tests:

- SLR: Negative bilaterally (improved from positive at 45°)
- Sensation: Improved - now intact L5 dermatome
- Motor: 5/5 EHL (improved from 4/5)

Functional:

- Oswestry: 28% (improved from 48%)
- Can sit 45 minutes
- Can lift 15 lbs safely

ASSESSMENT

Patient making good progress. Radicular symptoms largely resolved. Core strength improving. On track to meet goals.

PLAN

Continue PT 3x/week. Progress to more aggressive strengthening. Begin work conditioning activities. Coordinate with orthopedist regarding wrist recovery timeline.

Jennifer Martinez, DPT
Doctor of Physical Therapy
ProMotion Physical Therapy

Date: April 1, 2026